

ST. JUDE NEUROLOGICAL INSTITUTE & ADVANCED BRAIN CLINIC

Department of Clinical Neuroscience • Traumatic Brain Injury & Subdural Hematoma Evacuation Report

Patient Name:	Marcus Thorne	Patient Code / MRN:	PT-5519
Date of Birth:	1978-06-03 (Age 48)	Gender:	Male
Primary Diagnosis:	Right Acute-on-Chronic Subdural Hematoma	Attending Physician:	Dr. Sarah Jenkins, MD
Procedure Date:	2026-07-29	Surgical Unit:	Neuro-Surgical ICU

TRAUMA SUMMARY & SURGICAL INTERVENTION

48-year-old male involved in high-impact motor vehicle collision presenting with GCS 11 (E3V3M5) and unequal pupillary response. Non-contrast head CT revealed a 14 mm acute-on-chronic right hemispheric subdural hematoma with 7 mm midline shift and uncal herniation risk. Emergent right fronto-temporoparietal craniotomy and hematoma evacuation performed successfully with intracranial pressure (ICP) monitor placement.

POST-OPERATIVE NEUROLOGICAL COURSE

Post-operative ICP remained stable below 12 mmHg throughout ICU stay. Repeat CT scan at 48 hours confirmed complete hematoma decompression with resolution of midline shift. Patient extubated on post-op day 3. GCS improved to 15 (E4V5M6) with mild left pronator drift and transient memory processing lag.

LABORATORY & MEDICATION MANAGEMENT

- Prophylactic Levetiracetam 1000 mg BID initiated for early post-traumatic seizure prevention (target 7-day course).
- Coagulation Profile: INR 1.05, PTT 28 sec, Platelets 210 x10³/uL.
- Discharged to inpatient neuro-rehabilitation on post-op day 10.

PROGNOSIS & FOLLOW-UP

Excellent functional trajectory. Skull bone flap well-healed without sign of infection. Scheduled for follow-up outpatient CT and cognitive reassessment in 6 weeks.